

Guideline Title: Sepsis

Guideline Number: GUID-3203-M016

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To provide adequate airway management, hemodynamic support with fluid resuscitation and vasoactive medications as needed, as well as continue antibiotic treatment.
- II. **DEFINITIONS:**
 - a. MAP – mean arterial pressure
 - b. CVP – central venous pressure
 - c. PA Catheter – pulmonary artery catheter
 - d. PCWP – pulmonary capillary wedge pressure
 - e. SVR – systemic vascular resistance
 - f. CO – cardiac output
 - g. CI – cardiac index
- III. **DEPARTMENT GUIDELINE:**
 - a. Establish care as defined by *GUID3203-G002 – General Patient Care*.
 - b. Obtain history including recent lab work/meds. Note initial lactate.
 - c. Assess patient's core temperature if available, neurological status, and peripheral perfusion.
 - d. Assess hemodynamic status. If PA catheter is present, obtain or record last PCWP, SVR, CO/CI and monitor waveform.
 - e. If febrile, administer **Acetaminophen 10-15 mg/kg PO/PR** (Max 1,000mg) from sending facility.
 - f. If necessary, secure an advanced airway *per GUID3203-G008 Rapid Sequence Induction for Endotracheal Intubation and GUID3203-PR020 Endotracheal Intubation* and ventilate with a transport ventilator *per GUID3203-PR024 Ventilation with a Mechanical Ventilator*.
 - g. If hypoxia persists in an intubated/mechanically ventilated patient, consider adding PEEP to improve SpO2 unless patient is profoundly hypotensive.
 - h. Maintain MAP >65 mmHg and CVP > 8 mmHg (CVP > 12 mmHg if mechanically ventilated).
 - i. If signs of inadequate perfusion, begin resuscitation with 0.9% NS or Lactated Ringers *per GUID3203-M011 – Hypotension*. Initial volume resuscitation goals begin at 30 mL/kg in septic shock unless contraindicated.
 - ii. Monitor fluid intake and urine output if foley catheter is in place.

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iii. If CVP adequate/unknown, but MAP is low despite fluid resuscitation, start vasopressors
GUID3203-M011 – Hypotension.

1. **Norepinephrine infusion at 0.05-1 mcg/kg/min**, titrate q5 min to MAP > 65 mmHg. If BP not responsive to norepinephrine, consider adding:
2. **Epinephrine infusion at 0.05-2 mcg/kg/min**, titrate q5 min to MAP > 65 mmHg
3. **Vasopressin IV 0.03 units/min**
4. For hypotension refractory to vasopressors, consider **Hydrocortisone 100 mg IV** from sending facility

i. Broad spectrum antimicrobials should be administered and continued by the medical transport team. Consult with the sending/receiving physician regarding appropriate antimicrobials.

j. Verify blood glucose level. If <60mg/dL treat per *GUID3203-M021 – Hypoglycemia.*

IV. **DOCUMENTATION:**

- a. EMS Charts

V. **REFERENCES:**